

CLINICAL PRACTICE

Informed Consent for Psychotherapy (Adult)

A modern, plain-language consent form with optional modules and a state-law customization guide, updated for coverage, professional wills, consultation and AI.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Consent form + modules

VERSION

1.0 · Sept 2026

TIME TO ADAPT

45–60 minutes

WHAT IT IS

A comprehensive, plain-language informed consent form for adult outpatient psychotherapy, with optional modules and a checklist of state-specific items to confirm before use.

USE IT WHEN

You are opening a practice, refreshing your paperwork, or your current form lacks coverage, professional-will, consultation and AI language.

CUSTOMIZE FIRST

Mandated-reporting and duty-to-protect rules, minor-consent rules, record retention, recording laws, board contact language and your fee and cancellation terms.

How to use this form

The APA Ethics Code (Standard 10.01) expects psychologists to discuss, as early as feasible, the nature and anticipated course of therapy, fees, involvement of third parties and limits of confidentiality, and to give the patient an opportunity to ask questions. Use this form as the written record of that conversation, not a substitute for it. Read the key sections aloud in the first session.

CUSTOMIZE BEFORE YOU USE IT

Confidentiality exceptions, mandated reporting, duty to warn or protect, minors' consent, record retention and recording laws differ by state. The customization checklist at the end lists what to verify. Have an attorney or your liability carrier's risk-management service review the final version.

Informed consent for psychotherapy

Practice: [Practice name, address, phone]. **Clinician:** [Name, credential], licensed as a [psychologist / psychiatrist] in [state], License [number].

1. Welcome and purpose of this document

This document explains what to expect from therapy with me, how your information is protected, and the policies of my practice. Please read it carefully and ask me about anything that is unclear. You are welcome to take time before you sign.

2. About therapy

Psychotherapy is a collaborative process that can help with distress, relationships, behavior and life decisions. It works best when you take an active part. Benefits can include reduced distress, better relationships and greater clarity. Therapy can also involve risks: it may bring up painful feelings, and change in you can affect the people around you. Results vary and I cannot guarantee a particular outcome. There are alternatives, including medication, other forms of therapy, support groups and self-help resources, and I am glad to discuss them.

My approach is [brief description of approach and modalities]. We will work together to set goals, review progress regularly, and revise the plan when needed. You may end therapy at any time.

3. Sessions

Sessions are usually [45 to 53] minutes, [weekly] to start. The first [one to three] sessions are an assessment, after which we will discuss a treatment plan. My training and experience: [summary]. You may contact my licensing board with questions or concerns: [board name, phone, website].

4. Confidentiality and its limits

What you tell me is private and protected by law and by my professional ethics. I will not share it without your written permission, with these exceptions:

- **Safety.** If I believe you are at imminent risk of seriously harming yourself, I may take steps to protect you, including contacting emergency services or your emergency contact. If you communicate a serious threat to harm an identifiable person, I may be required or permitted by [state] law to warn or protect them.
- **Abuse and neglect.** I am required by [state] law to report known or reasonably suspected abuse or neglect of a child, and, where the law requires, an elder or dependent adult.
- **Legal process.** If a court orders me to disclose information, or a subpoena is served, I will assert privilege and protect your information as far as the law allows, and I will usually contact you first. I may be required to disclose in some circumstances.
- **Insurance and payment.** If you use insurance, I must provide the plan with information such as your diagnosis, dates of service and treatment information. If you do not pay, I may need to disclose limited information to a collections service after giving you notice.
- **Your own request.** I will share information with others only if you authorize it in writing.
- **Records and privilege.** Communications with me are generally privileged in legal settings, but there are exceptions, for example if you put your mental state at issue in a lawsuit.

5. Consultation, coverage and continuity

- **Consultation.** I sometimes consult with colleagues to improve your care. I do this without identifying you, unless you give consent.

- **Coverage.** When I am away, I may arrange for a colleague, [Name], to be available for urgent needs. They may receive the information needed for your safety and care.
- **If I cannot continue.** I have named [Name], a licensed [psychologist / psychiatrist], as my professional executor. If I become seriously ill, incapacitated or die, they may access my records to notify you, help you obtain your records, offer interim support if you need it, and help you find another qualified clinician.
- **Supervision.** [If applicable: I am supervised by (Name, credential), who may review my work and records.]

6. Contacting me and emergencies

I do not provide 24-hour emergency services. You can reach me at [phone or secure portal] and I aim to reply within [one business day]. Please do not use email or text for urgent matters or for sensitive clinical content. In an emergency call 911 or go to the nearest emergency room. For a mental health crisis, call or text 988, or chat at 988lifeline.org. [Optional: after-hours arrangement.]

I do not accept social media connection requests from patients, and I do not search for patients online without reason. If we meet in public, I will not acknowledge you unless you greet me first, to protect your privacy.

7. Telehealth and technology

If we meet by video or phone, please read and sign the Telehealth Consent. If I use any AI tool in connection with your care, I will explain it and ask for separate written consent.

8. Records

I keep a clinical record. You have the right to see and get a copy of it, and to ask for corrections, subject to the limits of federal and state law. Under HIPAA, [I keep / I do not keep] separate "psychotherapy notes," which have additional protection. I keep records for [number] years after our last contact, or longer if the law requires, and then dispose of them securely. My Notice of Privacy Practices explains your privacy rights in more detail.

9. Fees, insurance and cancellations

Fees, payment, insurance, missed sessions and the Good Faith Estimate are explained in my Financial Policy, which is part of this agreement.

10. Boundaries and professional relationship

Our relationship is professional. I will not enter into business, romantic or social relationships with you. If any dual relationship cannot be avoided, I will discuss it with you. Please tell me if you have concerns about our work together, and we will address them. If we cannot resolve them, you may contact my licensing board.

11. Ending therapy

You can stop at any time. I recommend we discuss ending in a session so we can review progress and plan. I may end therapy if I believe you are no longer benefiting, if you need a different level of care, if payment is not made as agreed, or if the relationship cannot continue safely. If so, I will explain, give referrals, and help with transition.

12. Consent

I have read and understand this document and have had the chance to ask questions. I agree to begin psychotherapy with [Clinician name]. I understand that I can withdraw consent at any time.

Patient signature

Date

Clinician signature

Date

Optional modules

Module A: Couples and family therapy

In couples or family therapy, the couple or family is the patient. Information shared with me by one member individually may be shared with the others if it is relevant to the work. [Policy on secrets: I will not agree to keep secrets that harm the therapy.] Each adult must sign this form. Each person's records may be released only with the written consent of each adult participant, subject to law. Therapy records may not be usable for custody or divorce proceedings, and I will not act as an expert witness or custody evaluator for participants.

Module B: Minors (with parent or guardian)

The patient is a minor. Parents or guardians consent to treatment, and are generally entitled to information about the minor's care, subject to state law. To help the child feel safe to talk, we agree that I will share general progress, safety concerns and treatment plans, and I will not share the details of what the child says unless there is a safety concern or the child agrees. Where [state] law allows minors of [age] to consent to treatment on their own, different rules may apply. In cases of separated or divorced parents, I need a copy of custody or decision-making orders before treatment begins.

Module C: Psychological assessment or testing

Assessment involves interviews, questionnaires and standardized tests. I will explain the purpose, who will receive the report, and how the results may be used. Before I provide results to anyone else, I need your written authorization, unless the law requires otherwise. If the evaluation is for a third party (an employer, school or court), I do not have a therapeutic relationship with you and the usual confidentiality protections do not apply in the same way. I will explain who receives the report.

Module D: Group therapy

Group members agree to keep what others say confidential. I cannot guarantee that members will do so, and the law may not protect statements made in group. Members do not socialize outside group in ways that harm the group's work. Members attend regularly and arrive on time. I may end a member's participation if their behavior harms the group.

Customization checklist

Check each with your state board, professional association or attorney.

- ☐ Mandated reporting: who is a mandated reporter, what must be reported, timeframes, and elder/dependent adult reporting.
- ☐ Duty to warn or protect: mandatory, permissive or none, and how it applies to telehealth patients in other states.
- ☐ Age and rules for minors' consent to mental health treatment and parents' access.
- ☐ Disclosure requirements: license number, board contact, supervisor disclosure, fee schedule notice.
- ☐ Record retention period and requirements for minors.
- ☐ Recording, wiretap and one-party versus all-party consent laws.
- ☐ Privilege and testimony, including limitations in custody and civil cases.
- ☐ Telehealth licensure and consent rules, and PSYPACT participation.
- ☐ Payer requirements for consent language and notice of network status.
- ☐ Language and accessibility: provide forms in the patient's language and in accessible formats.

IN PSYALLIANCE Add this form to your personal library and adapt it. Use it with the Telepsychology Consent, AI Tools Policy, Financial Policy and Privacy Notice templates so patients receive one coherent onboarding set.

Sources and further reading

- APA Ethics Code, Standards 4.02, 4.05, 4.06, 6.04 and 10.01 ([apa.org/ethics/code](https://www.apa.org/ethics/code)).
- APA Practice Organization, Informed consent guidance and templates for psychologists ([apaservices.org](https://www.apaservices.org)).
- HHS, HIPAA Privacy Rule and Notice of Privacy Practices ([hhs.gov/hipaa](https://www.hhs.gov/hipaa)).

ABOUT THIS DOCUMENT

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